Eribulin · Days 1 and 15 of 28

NCCP 00743 · Source reconciled; independent clinical, pharmacy and patient review pending

PRACTICAL TREATMENT GUIDE

Your infusion and access plan

Eribulin on Days 1 and 15 of each 28-day cycle. Days 8 and 22 are planned treatment-free weeks. This alternative schedule requires a specific discussion and consent.

STEP 1

Before the first visit

Confirm the indication, baseline bloods, required cardiac checks, medicines review and consent before the first infusion.

STEP 2

Plan venous access

A cannula, PICC or implanted port may be used. If a port is chosen, a trained team inserts it in a separate procedure with local anaesthetic and sometimes sedation; the team explains fasting, blood-thinner and wound instructions.

STEP 3

Infusion day

Eribulin is given by IV infusion over 5 minutes; the total appointment includes checks and preparation. Staff observe for reactions. Report burning, pain, swelling or leakage at the infusion site immediately.

STEP 4

Home and next review

This regimen does not routinely need a take-home infusion pump. Staff disconnect the infusion; a port is de-accessed when appropriate. Keep line-care and next-visit instructions.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Visit length

Eribulin is given by IV infusion over 5 minutes; the total appointment includes checks and preparation. A port is optional access, not a requirement for a take-home pump. This regimen has no routine ambulatory pump.

Premedication

No routine hypersensitivity premedication is specified. Low-risk antiemetic support is reviewed separately, including QT interactions.

Sources: NCCP 00743 v3 · official source · eviQ · implanted port patient information · eviQ · PICC patient information. Source reconciled; independent clinical, pharmacy and patient review pending.